

SELF-ADVOCACY 101

Human Dignity, Systemic Harm, and Institutional Neglect

A plain-language vocabulary and first-person analysis of dignity, barriers,
fragmented care, and meaningful access.

Thomas Harrison / Honestly Thomas
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Defining Terms of Human Dignity

A working vocabulary for describing rights, relationships, support, control, and the conditions people need to participate in their own lives.

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Agency

Your ability to make choices and act on them.

Autonomy

Control over your own body, life, and decisions.

Independence

Living without unnecessary control or forced reliance.

Interdependence

People supporting each other without taking away autonomy.

Support

Help that increases your ability to live your own life.

Protection

Preventing harm without becoming controlling.

Safety

Freedom from danger, coercion, exploitation, and abandonment.

Dignity

Being treated as a person of equal human worth.

Sovereignty

Rightful authority over yourself.

Consent

Freely agreeing with adequate information and a real choice.

Capacity

Your present ability to understand and make a particular decision.

Accommodation

Changing a process so disability does not block access.

Accessibility

Making participation genuinely possible.

Advocacy

Communicating and defending someone's needs, choices, and rights.

Self-Advocacy

Representing your own needs and lived reality.

Supported Decision-Making

Receiving help with decisions without surrendering control.

Continuity

Support that does not disappear between appointments or referrals.

Accountability

Responsibility for actions, failures, and outcomes.

Transparency

Clearly explaining decisions, processes, and limitations.

Validation

Recognizing someone's experience as real.

Trust

Confidence built through honesty and consistent action.

Boundaries

Limits that protect a person's time, body, energy, and identity.

Neglect

Failing to provide necessary care or protection.

Abandonment

Withdrawing support while significant needs remain unresolved.

Dependency

Relying on something or someone to meet essential needs.

Empowerment

Increasing someone's choices, confidence, and control.

Community

Reciprocal human connection and belonging.

Inclusion

Being welcomed as a participant, not merely managed as a problem.

Humanity

The whole person beyond diagnoses, records, risks, and services.

Language does not create human dignity. It gives people tools to identify when dignity is being supported, restricted, or denied.

Recognizing Barriers and Systemic Harm

A vocabulary for identifying the progression from dehumanizing outcomes back through institutional mechanisms - and returning authority to lived experience.

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Dehumanization

Treating a person as a problem to manage instead of a human being with dignity.

Systemic Neglect

Repeated harm produced by the combined failures of policies, institutions, and fragmented responsibility.

Institutional Betrayal

Being harmed by an organization that was expected to provide safety or support.

Abandonment

Withdrawing involvement while significant needs remain unresolved.

Neglect

Failing to provide necessary care, protection, or follow-through.

Retraumatization

Recreating the fear, helplessness, or loss of control caused by earlier trauma.

Administrative Burden

Forcing someone to perform excessive paperwork, coordination, and follow-up to obtain help.

Scapegoating

Making one person carry the blame for a larger systemic failure.

Blame-Shifting

Transferring responsibility from the person or institution causing harm onto the person experiencing it.

Doublespeak

Using reassuring language to disguise harmful actions or absent support.

Gaslighting

Repeatedly manipulating someone into doubting their own memory, perception, or reality.

Gatekeeping

Controlling access to support through unnecessary barriers or subjective judgments.

Discrimination

Unequal treatment based on identity, disability, or another protected characteristic.

Ableism

Treating disabled people as less capable, credible, or valuable.

Stigma

Reducing someone to negative assumptions about their identity or diagnosis.

Tone Policing

Focusing on someone's emotional delivery instead of what they are reporting.

Minimization

Presenting serious harm as less important than it is.

Invalidation

Denying or rejecting the reality of someone's experience.

Dismissal

Treating someone's concern as unworthy of serious attention.

Lived Experience

Knowledge gained by directly living through something.

Recognizing systemic harm is not pessimism. It is how lived experience becomes evidence instead of something institutions can quietly explain away.

Why Institutional Neglect Is Psycho-Spiritual Warfare

How fragmented care converts requests for help into invisible labour, erodes human dignity, and mistakes procedural activity for meaningful access.

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Abstract

This paper uses “psycho-spiritual warfare” as a metaphor for the cumulative inner damage caused by prolonged institutional neglect. It argues that fragmented mandates, unsupported referrals, repeated retelling, and absent accountability can attack a person’s trust in their own perception, their relationship with hope, and their sense of belonging in the human world. The proposed remedy is neither heroic nor complicated: continuity, warm handoffs, shared responsibility, and access to ordinary community beyond the care industry.

Defining the term

When I call institutional neglect “psycho-spiritual warfare,” I am not claiming there is a supernatural conspiracy against me. I am describing what prolonged, organized indifference does to a human being from the inside.

It attacks your ability to trust your own perception. It damages your relationship with hope. It makes every attempt to ask for help feel like volunteering to be misunderstood again. Eventually, it reaches beyond your circumstances and starts interfering with your sense of identity, dignity, meaning, and belonging.

That is what I mean by psycho-spiritual warfare.

How fragmentation becomes abandonment

It rarely begins with one dramatic act of cruelty. Usually, it begins with fragmentation.

One institution handles income. Another handles housing. Another handles mental health. Another handles developmental disability. Another handles legal questions. Every organization has its own intake process, eligibility requirements, communication style, waiting list, and narrow definition of what it is responsible for.

Each person sees one page of your life. Nobody reads the whole book.

You explain that you are isolated and cannot navigate the system alone. They respond by giving you another telephone number.

You contact that number. Nobody answers, or you leave a message and hear nothing meaningful back. You email. You wait. You follow up. Then you contact another organization and explain that the first one did not respond.

That organization tells you to contact the first one again.

- 1 Reach out and disclose the need.
- 2 Receive a referral instead of continuity.
- 3 Encounter silence, delay, or another intake barrier.
- 4 Report that failure to a different institution.
- 5 Get redirected to the original doorway.
- 6 Restart alone.

Now you are not merely seeking support. You are gathering evidence that you sought support. You are documenting unanswered calls, preserving emails, translating your life into institutional language, and preparing to defend yourself against the future accusation that you did not try hard enough.

The entire burden of coordination has been assigned to the person who explicitly said they cannot coordinate it alone.

The illusion of available help

Then somebody says, “Help is available. You just have to reach out.”

I have reached out.

I have contacted disability services, mental-health services, income-support workers, community organizations, oversight bodies, and legal services. I have explained my disabilities, my isolation, my need for advocacy, and the barriers preventing me from completing yet another unsupported intake.

The result is often another referral back into the same network.

The referral gets counted as assistance. The unanswered referral does not get counted as failure.

On paper, every organization may have completed its task. Someone provided a phone number. Someone sent a link. Someone explained that my needs fall outside their mandate. Someone recommended an agency that appears, from its website, to provide exactly what I need.

The paperwork shows movement. My life does not.

The invisible trauma

That gap is where invisible trauma accumulates.

The trauma is not only that help failed to arrive. It is that I must repeatedly transform my suffering into an acceptable administrative format.

I have to sound distressed enough to communicate urgency, but calm enough not to be dismissed as unstable. I have to explain the effects of my disabilities while proving that I am competent enough to deserve autonomy. I have to describe years of institutional failure without sounding angry at institutions.

Every sentence becomes a tiny courtroom where I am both the witness and the accused.

When I communicate clearly, people assume I am functioning well enough to continue alone. When I become overwhelmed, that overwhelm can be treated as the problem instead of evidence of what the process has done to me.

Competence becomes evidence that I do not need help. Collapse becomes evidence that I am too difficult to help.

That is a closed system.

Psychological, social, and spiritual injury

After enough repetitions, the damage becomes psychological. You begin anticipating rejection before asking. You reread every message, trying to remove anything that could be misunderstood or weaponized. You spend hours finding the exact language required to make another person recognize something you already experience as painfully obvious.

Then the damage becomes social. Your family sees you sending emails and assumes the situation is being handled. Professionals see a list of referrals and assume you are connected to services. Each person knows the storybook version: the disabled person has agencies, doctors, programs, and family somewhere around them, so surely somebody must be helping.

But everybody is standing around the empty space where the actual help should be.

Then the damage becomes spiritual - not necessarily in a religious sense, but at the level where a person asks:

Do I belong in the human world?
Does my experience become real only after an institution validates it?
Am I a person with something to contribute, or merely a collection of symptoms, risks, and eligibility categories to be managed?

This is why I use the word “warfare.” Not because every worker is malicious. Many are probably exhausted people trying to do something decent inside systems built around limited mandates and defensive procedures.

But harm does not require a mastermind. When responsibility is divided thinly enough, everybody can follow procedure while one person is gradually destroyed by the combined outcome.

Natural consequences, not cosmic punishment

That outcome has consequences. Not biblical punishment. Not karma keeping score. Cause and effect.

When human dignity is repeatedly displaced, suffering emerges from the conditions. Isolation produces despair. Repeated dismissal produces distrust. Forced dependency weakens agency. Chronic instability makes ordinary tasks harder. Then those predictable effects are treated as personal defects.

The system creates the conditions of collapse and later cites the collapse as justification for more control.

That is the warfare: a person must fight to preserve an inner sense of reality while every external process reduces that reality to somebody else’s jurisdiction.

The simple solution: continuity

So what is the solution?

It is embarrassingly simple.

One person stays.

Not forever. Not as a saviour. Not as another authority assigned to monitor and contain somebody.

One person listens to the whole situation, helps determine the next step, makes a warm connection to the appropriate organization, and remains involved until that connection actually succeeds.

Continuity - Keep one accountable human connection across institutional boundaries.

Warm handoff - Transfer responsibility person-to-person, not person-to-phone-number.

Follow-through - Confirm contact, accommodation, ownership, and the next step.

Community - Support ordinary belonging outside professional care relationships.

A referral is not complete when a phone number is given. It is complete when contact has occurred, accommodations are in place, responsibility has been accepted, and the person knows what happens next.

If the referral fails, the referring organization should help repair it. The disabled person should not be dropped back at the beginning of the maze.

Beyond the care industry

Beyond professional continuity, people need community outside the care industry.

I do not only need more efficient management of my disability. I need ordinary human belonging: someone who knows me because they share life with me, not because they were assigned my file. Someone who sees my ideas, humour, work, and potential - not only my needs.

Professional support should help make that independence and community possible. It should not become a permanent substitute for them.

A different measure of care

The solution is not another list of resources. It is shared responsibility, a warm handoff, meaningful follow-through, and one human being refusing to disappear the moment the conversation becomes inconvenient.

Believe people before they collapse.

Treat repeated unanswered outreach as a failure of access, not a failure of motivation.

Stop measuring assistance by whether information was provided. Measure it by whether the person became less trapped.

If every institution can say, "We did our part," while the human being remains completely alone, then the system is not providing care. It is distributing responsibility until nobody can be held responsible for the suffering left behind.